250182514842231

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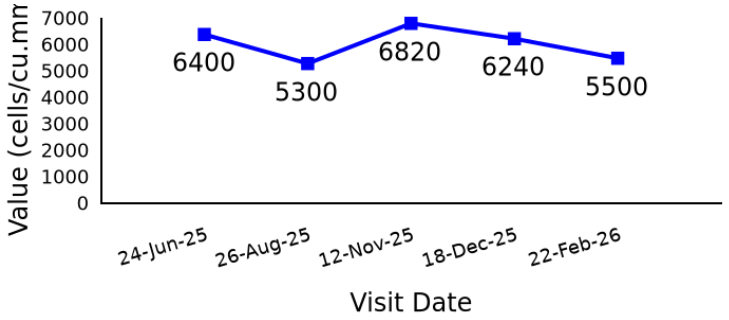
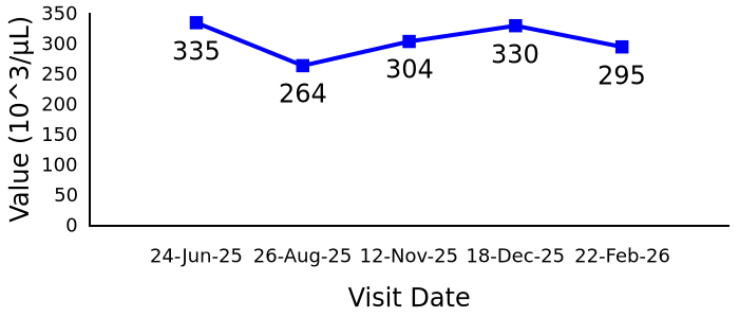
## Result Trend (For selected tests used for followup)

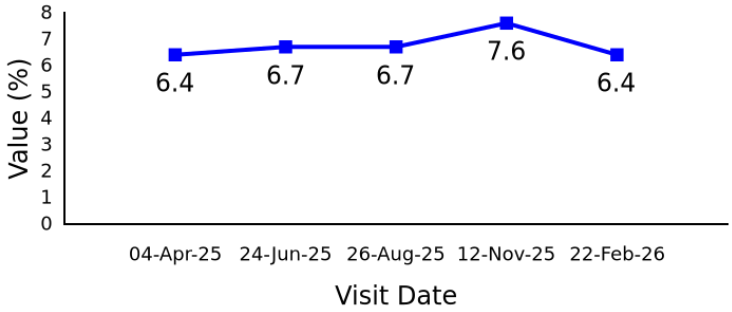
### Total Leucocytes (WBC) Count



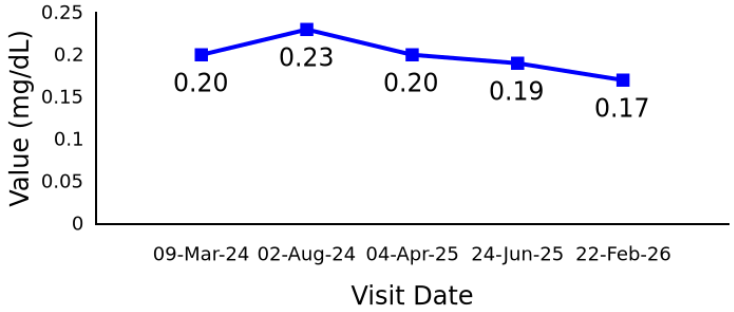

### Platelet count



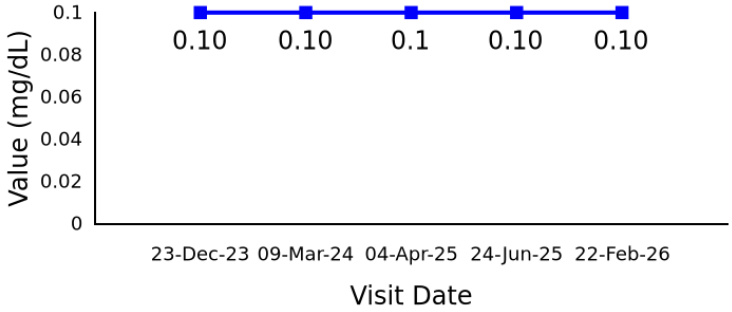

### HbA1C- Glycated Haemoglobin



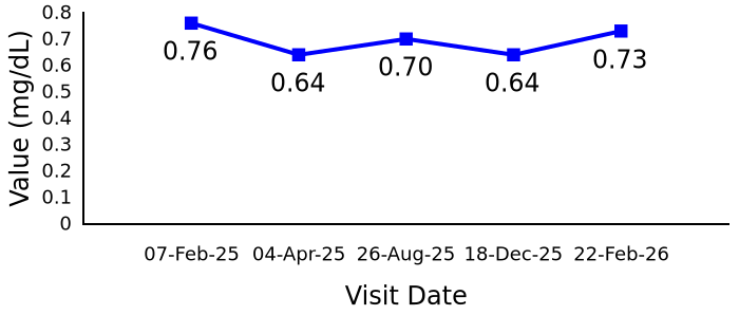

### Bilirubin Total




### Bilirubin Direct




### Creatinine, Serum



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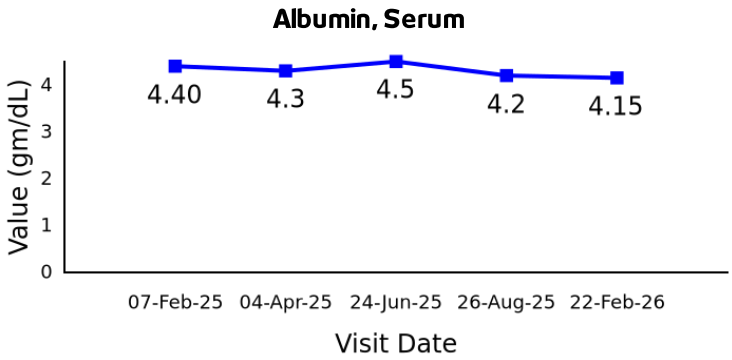


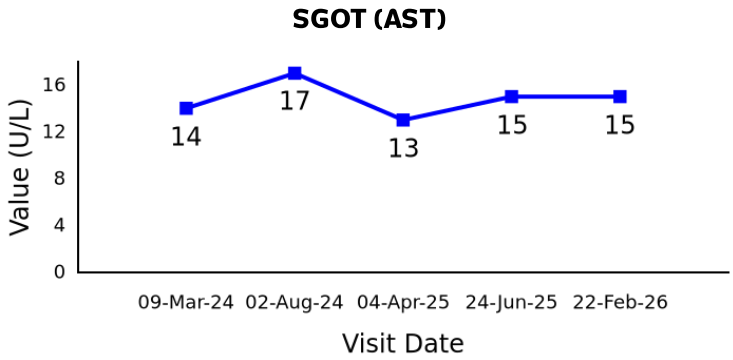


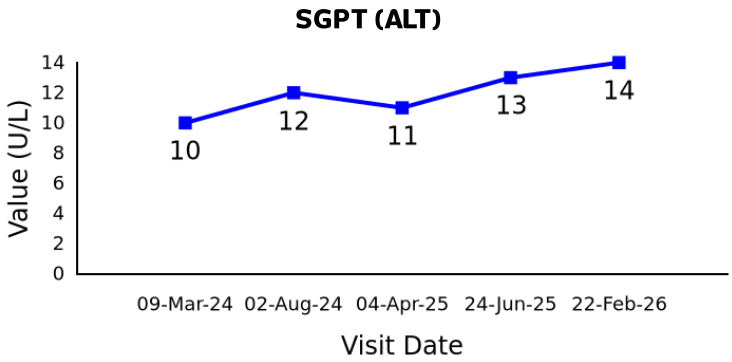


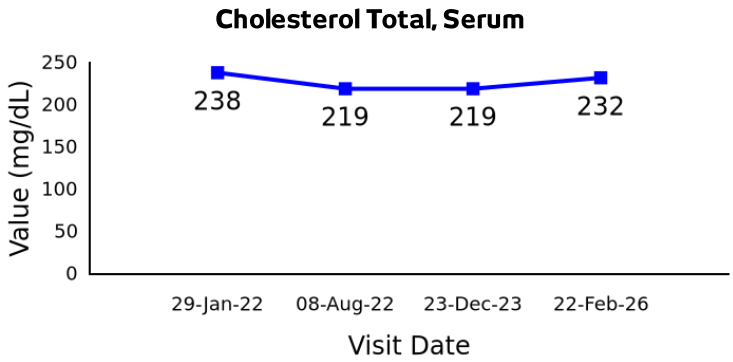


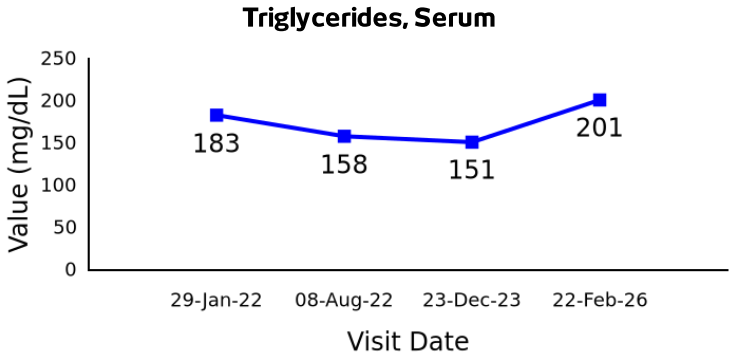


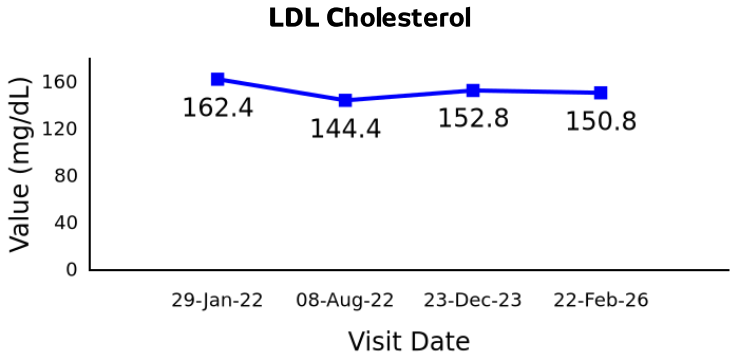




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## HbA1c- Glycated Haemoglobin
## (EDTA Whole Blood )

| Investigation | Observed Value | Unit | Biological Reference Interval |
|---|---|---|---|
| **HbA1C- Glycated Haemoglobin**<br>(High-Performance Liquid Chromatography (HPLC)) | **6.4** | % | Non-diabetic: <= 5.6<br>Pre-diabetic: 5.7-6.4<br>Diabetic: >= 6.5 |
| **Estimated Average Glucose (eAG)** | 137 | mg/dL | |

**Interpretation & Remark:**

1. HbA1c is used for monitoring diabetic control. It reflects the estimated average glucose (eAG).
2. HbA1c has been endorsed by clinical groups & ADA (American Diabetes Association) guidelines 2017, for diagnosis of diabetes using a cut-off point of 6.5%.
3. Trends in HbA1c are a better indicator of diabetic control than a solitary test.
4. Low glycated haemoglobin(below 4%) in a non-diabetic individual are often associated with systemic inflammatory diseases, chronic anaemia(especially severe iron deficiency & haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.
5. To estimate the eAG from the HbA1C value, the following equation is used: eAG(mg/dl) = 28.7*A1c-46.7
6. Interference of Haemoglobinopathies in HbA1c estimation.

   A. For HbF > 25%, an alternate platform (Fructosamine) is recommended for testing of HbA1c.
   B. Homozygous hemoglobinopathy is detected, fructosamine is recommended for monitoring diabetic status
   C. Heterozygous state detected (D10/ turbo is corrected for HbS and HbC trait).

7. In known diabetic patients, following values can be considered as a tool for monitoring the glycemic control. Excellent Control - 6 to 7 %, Fair to Good Control - 7 to 8 %, Unsatisfactory Control - 8 to 10 % and Poor Control - More than 10 % .

Note : Hemoglobin electrophoresis (HPLC method) is recommended for detecting hemoglobinopathy.

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## CBC Haemogram

| Investigation | Observed Value | Unit | Biological Reference Interval |
|---|---|---|---|
| ***Erythrocytes*** | | | |
| **Haemoglobin (Hb)** (Photometric measurement) | **12.1** | gm/dL | 12.5-16 |
| **Erythrocyte (RBC) Count** (RBC Pulse Height Detection method) | 4.28 | mill/cu.mm | 4.2-5.4 |
| **PCV (Packed Cell Volume)** (Calculated) | **36.0** | % | 37-47 |
| **MCV (Mean Corpuscular Volume)** (Automated - Pulse Height Detection) | 84.1 | fL | 78-100 |
| **MCH (Mean Corpuscular Hb)** (Calculated) | 28.2 | pg | 27-31 |
| **MCHC (Mean Corpuscular Hb Concn.)** (Calculated) | 33.5 | gm/dL | 32-36 |
| **RDW (Red Cell Distribution Width)** (Derived from RBC Histogram) | 14.0 | % | 11.5-14.0 |
| ***Leucocytes*** | | | |
| **Total Leucocytes (WBC) Count** (Automated Cell Counter) | 5500 | cells/cu.mm | 4000-10500 |
| **Absolute Neutrophils Count** (Calculated) | 2750 | cells/cu.mm | 2000-7000 |
| **Absolute Lymphocyte Count** | 2156 | cells/cu.mm | 1000-3000 |
| **Absolute Monocyte Count** (Calculated) | 435 | cells/cu.mm | 200-1000 |
| **Absolute Eosinophil Count** (Calculated) | 94 | cells/cu.mm | 20-500 |
| **Absolute Basophil Count** (Calculated) | 66 | cells/cu.mm | 20-100 |
| **Neutrophils** (Automated Cell Counter) | 50.0 | % | 40-80 |
| **Lymphocytes** (Automated Cell Counter) | 39.2 | % | 20-40 |
| **Monocytes** (Automated Cell Counter) | 7.9 | % | 02-10 |

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| Investigation | Observed Value | Unit | Biological Reference Interval |
|---|---|---|---|
| **Eosinophils** (Automated Cell Counter) | 1.7 | % | 1-6 |
| **Basophils** (Automated Cell Counter) | 1.2 | % | 0-2 |
| ***Platelets*** | | | |
| **Platelet count** (Automated - Pulse Height Detection) | 295 | 10^3/µL | 150-450 |
| **MPV (Mean Platelet Volume)** | 8.2 | fL | 6-9.5 |
| **PCT ( Platelet Haematocrit)** (Calculated) | 0.241 | % | 0.2-0.5 |
| **PDW (Platelet Distribution Width)** (Calculated) | 16.3 | % | 9-17 |

EDTA Whole Blood - Tests done on Automated Five Part Cell Counter. (WBC,Platelet count by impedance method/DC detection,RBC by pulse height detection method, HB by Automated - Photometric Measurement, WBC differential by VCS technology other parameters calculated)

**All Abnormal Haemograms are reviewed confirmed microscopically.**

Differential count is based on approximately 10,000 cells.

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| Investigation | Observed Value | Unit | Biological Reference Interval |
|---|---|---|---|
| **Creatinine, Serum**<br>(Serum,Jaffes method) | 0.73 | mg/dL | 0.5-0.9 |

**Interpretation -**

Creatinine is a waste product formed in muscles from the high energy storage compound, creatine phosphate. The amount of creatinine produced is constant (unlike Urea) and is primarily a function of muscle mass. Physiological factors affecting serum creatinine concentration includes age, gender, race, muscularity, exercise, Pregnancy, certain drugs, diet, dehydration and nutritional status. Low serum Creatinine levels is seen in cases of low muscle mass like muscular atrophy, or aging. High serum creatinine levels is seen in Acute and Chronic kidney disease, obstruction. Since a rise in blood creatinine is observed only with marked damage of the nephrons, it is not suited to detect early stage kidney disease.

| | | | |
|---|---|---|---|
| **Uric Acid, Serum**<br>(Serum,Uricase) | 4.6 | mg/dL | 2.4-5.7 |

**Interpretation:**

- Increased in Gout, asymptomatic hyperuricemia, leukemia, polycythemia, hemolytic anemia, sickle cell anemia, resolving pneumonia, toxemia of pregnancy, psoriasis, lymphoma, metabolic acidosis, chronic lead poisoning.
- Decreased in disorders of copper accumulation , kidney tubule disorder, Acromegaly, Celiac disease, Xanthine oxidase deficiency.
- Its used to monitor gout and also chemotherapeutic treatment of neoplasm to avoid renal urate deposition with possible renal failure (tumor lysis syndrome).

**Note:**

- A purine rich diet as well as sever exercise increases uric acid values.
- High protein-weight reduction diet and alcohol consumption can cause raised uric acid levels.

**Reeference:**


- Package insert
- Wallach's interpretation of diagnostic tests, Ed11, 2020.
- Henry's Clinical Diagnosis and Management by Laboratory Methods. 23rd ed; 2017.
- Tietz fundamentals of clinical chemistry 6th edition. Burtis CA, Ashwood ER, Bruns DE, 2008.


***Lipid Profile - 2 (Mini - Fasting)***

| | | | |
|---|---|---|---|
| **Cholesterol Total, Serum**<br>(Serum,Cholesterol oxidase-peroxidase (CHOD-POD)) | **232** | mg/dL | Desirable: < 200<br>Borderline High: 200-239<br>High: >= 240 |

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| Investigation | Observed Value | Unit | Biological Reference Interval |
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| **Triglycerides, Serum**<br>(Serum,Glycerol-3-phosphate peroxidase chromogenic method (GPO-POD)) | **201** | mg/dL | Normal: < 150<br>Borderline High: 150-199<br>High: 200-499<br>Very High: >= 500 |
| **HDL Cholesterol Direct**<br>(Serum,Direct Homogeneous method) | 41 | mg/dL | > 40 |
| **Non HDL Cholesterol**<br>(Serum,Calculated) | **191.00** | mg/dL | Optimal <130<br>,Desirable 130-159<br>,Borderline high 160-189<br>,High 190-220<br>,Very High >=220 |
| **LDL Cholesterol**<br>(Serum,Calculated) | 150.8 | mg/dL | Optimal <100<br>,Near Optimal 100-129<br>,Borderline high 130-159<br>,High 160-189<br>,Very High >=190 |
| **VLDL Cholesterol**<br>(Serum,Calculated) | **40.20** | mg/dL | 6-38 |
| **LDL/HDL Ratio**<br>(Serum,Calculated) | 3.68 | | 2.5-3.5 |
| **Cholestrol / HDL Ratio**<br>(Serum,Calculated) | 5.66 | | 3.5-5.0 |

**Note:** Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.

***LFT (Liver Function Test) 2 - Mini***

***Bilirubin Total, Direct, Indirect***

| | | | |
|---|---|---|---|
| **Bilirubin Total**<br>(Serum,Diazo method) | **0.17** | mg/dL | 0.2-1.2 |
| **Bilirubin Direct**<br>(Serum,Diazo method) | 0.10 | mg/dL | <= 0.30 |
| **Bilirubin- Indirect**<br>(Serum,Calculated) | **0.07** | mg/dL | 0.1-1.0 |
| **SGOT (AST)**<br>(Serum,Enzymatic) | 15 | U/L | 0-31 |

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| **SGPT (ALT)**<br>(Serum,Enzymatic) | 14 | U/L | 0-34 |
| **Alkaline Phosphatase, Serum**<br>(Serum,p-nitrophenyl phosphate (pNPP)) | 83 | U/L | 35-105 |
| **Gamma GT (GGTP)**<br>(Serum,Enzymatic) | 13 | U/L | 5-36 |
| ***Proteins, Serum*** | | | |
| **Total Protein**<br>(Serum,Biuret test) | 7.11 | gm/dL | 6.4-8.3 |
| **Albumin, Serum**<br>(Serum,Bromocresol Green (BCG)) | 4.15 | g/dL | 3.5-5.2 |
| **Globulin**<br>(Serum,Calculated) | 2.96 | gm/dL | 1.8-3.6 |
| **Albumin/Globulin Ratio**<br>(Serum,Calculated) | 1.40 | | 1.1-2.2 |

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| Investigation | Observed Value | Unit | Biological Reference Interval |
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| ***Haemogram Advanced Profile - 1*** | | | |
| **ESR (Erythrocyte Sedimentation Rate)**<br>(EDTA Whole Blood,Capillary Photometry) | 6 | mm/hr | 0-20 |

**Interpretation:**

- It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
- It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
- It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

**Remark :** ESR Performed using capillary photometric aggregation (for automated analysis) & westergrens (for manual testing).

**-- End of Report --**

MC-2518

Test Marked with NABL symbol are in the scope of accrediation

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